

1. NAME OF THE MEDICINAL PRODUCT

IZBA 30 micrograms / ml eye drops, solution

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

1 ml of solution contains 30 micrograms of travoprost.

Preservative: 1 ml of solution contains 10 micrograms of polyquaternium-1.

Excipients with known effect: 1 ml of solution contains 7.5 mg propylene glycol and 2 mg polyoxyethylene hydrogenated castor oil 40 (HCO-40) (see section 4.4).

For the full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Eye drops, solution.

Clear, colourless to light yellow solution.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

IZBA eye drops contains travoprost, a prostaglandin analogue.

IZBA eye drops is indicated for the decrease of elevated intraocular pressure in adult patients with ocular hypertension or open-angle glaucoma (see section 5.1).

4.2 Posology and method of administration

Posology

Use in adults, including elderly patients

The dose is 1 drop of IZBA eye drops in the conjunctival sac of the affected eye(s) once daily. Optimal effect is obtained if the dose is administered in the evening.

If a dose is missed, treatment should be continued with the next dose as planned. The dose should not exceed 1 drop in the affected eye(s) daily.

When substituting another ophthalmic antiglaucoma medicinal product with IZBA eye drops, the other medicinal product should be discontinued and IZBA eye drops should be started the following day.

Use in patients with hepatic or renal impairment

Travoprost 30 µg / ml has not been studied in patients with hepatic or renal impairment. However, travoprost 40 µg / ml has been studied in patients with mild to severe hepatic impairment and in patients with mild to severe renal impairment (creatinine clearance as low as 14 ml / min). No dosage adjustment is necessary in these patients (see section 5.2).

Therefore, no need for dose adjustment at the lower concentration of active ingredient is anticipated.

Use in children and adolescents

The safety and efficacy of IZBA eye drops in children and adolescents below the age of 18 years has not been established.

No data are available.

Method of administration

For ocular use.

For patients who wear contact lenses, patients must be instructed to remove contact lenses prior to application of IZBA eye drops and wait at least 15 minutes after instillation of the dose before reinsertion.

The patient should remove the protective overwrap immediately prior to initial use.

To prevent contamination of the dropper tip and solution, care must be taken not to touch the eyelids, surrounding areas or other surfaces with the dropper tip of the bottle. Keep the bottle tightly closed when not in use.

Nasolacrimal occlusion or gently closing the eyelid after administration is recommended. This may reduce the systemic absorption of medicinal products administered via the ocular route and result in a decrease in systemic adverse reactions.

If more than one topical ophthalmic medicinal product is being used, the medicinal products must be administered at least 5 minutes apart. Eye ointments should be administered last.

4.3 Contraindications

Hypersensitivity to the active substance or to any of the excipients listed in section 6.1.

4.4 Special warnings and precautions for use

Eye colour change

Travoprost may gradually change the eye colour by increasing the number of melanosomes (pigment granules) in melanocytes. Before treatment is instituted, patients must be informed of the possibility of a permanent change in eye colour. Unilateral treatment can result in permanent heterochromia. The long term effects on the melanocytes and any consequences thereof are currently unknown. The change in iris colour occurs slowly and may not be noticeable for months to years. The change in eye colour has predominantly been seen in patients with mixed coloured irides, i.e., blue-brown, grey-brown, yellow-brown and green-brown; however, it has also been observed in patients with brown eyes. Typically, the brown pigmentation around the pupil spreads concentrically towards the periphery in affected eyes, but the entire iris or parts of it may become more brownish. After discontinuation of therapy, no further increase in brown iris pigment has been observed.

Periorbital and eye lid changes

In controlled clinical trials, periorbital and / or eyelid skin darkening in association with the use of travoprost has been reported in 0.2 % of patients.

Periorbital and lid changes including deepening of the eyelid sulcus have been observed with prostaglandin analogues.

Travoprost may gradually change eyelashes in the treated eye(s); these changes were observed in about half of the patients in clinical trials and include : increased length, thickness, pigmentation, and / or number of lashes. The mechanism of eyelash changes and their long term consequences are currently unknown.

There is no experience of travoprost in inflammatory ocular conditions; nor in neovascular, angle-closure, narrow-angle or congenital glaucoma and only limited experience in thyroid eye disease, in open-angle glaucoma of pseudophakic patients and in pigmentary or pseudoexfoliative glaucoma.

Aphakic patients

Caution is recommended when using travoprost in aphakic patients, pseudophakic patients with a torn posterior lens capsule or anterior chamber lenses, or in patients with known risk factors for cystoid macular oedema.

Iritis / uveitis

In patients with known predisposing risk factors for iritis / uveitis, travoprost can be used with caution.

Contact with the skin

Skin contact with travoprost must be avoided as transdermal absorption of travoprost has been demonstrated in rabbits.

Prostaglandins and prostaglandin analogues are biologically active materials that may be absorbed through the skin. Women who are pregnant or attempting to become pregnant should exercise appropriate precautions to avoid direct exposure to the contents of the bottle. In the unlikely event of coming in contact with a substantial portion of the contents of the bottle, thoroughly cleanse the exposed area immediately.

Excipients

IZBA eye drops contains propylene glycol which may cause skin irritation.

IZBA eye drops contains polyoxyethylene hydrogenated castor oil 40 which may cause skin reactions.

4.5 Interaction with other medicinal products and other forms of interaction

No interaction studies have been performed.

4.6 Fertility, pregnancy and lactation

Women of child-bearing potential / contraception

IZBA eye drops must not be used in women of child bearing age / potential unless adequate contraceptive measures are in place (see section 5.3).

Pregnancy

Travoprost has harmful pharmacological effects on pregnancy and / or the foetus / new-born child.

IZBA eye drops should not be used during pregnancy unless clearly necessary.

Breast-feeding

It is unknown whether travoprost from eye drops is excreted in human breast milk. Animal studies have shown excretion of travoprost and metabolites in breast milk.

The use of IZBA eye drops by breast-feeding mothers is not recommended.

Fertility

There are no data on the effects of travoprost on human fertility. Animal studies showed no effect of travoprost on fertility at doses more than 250 times the maximum recommended human ocular dose.

4.7 Effects on ability to drive and use machines

IZBA eye drops has no or negligible influence on the ability to drive and use machines.

Temporary blurred vision or other visual disturbances may affect the ability to drive or use machines. If blurred vision occurs at instillation, the patient must wait until the vision clears before driving or using machines.

4.8 Undesirable effects

Summary of the safety profile

In a clinical trial of 3 months duration (N = 442) involving IZBA eye drops as monotherapy, the most common adverse reaction observed was hyperaemia of the eye (ocular or conjunctival) reported in approximately 12 % of the patients.

Tabulated list of adverse reactions

The following adverse reactions were assessed to be related with IZBA eye drops monotherapy and are classified according to the following convention : very common ($\geq 1 / 10$), common ($\geq 1 / 100$ to $< 1 / 10$), uncommon ($\geq 1 / 1,000$ to $< 1 / 100$), rare ($\geq 1 / 10,000$ to $< 1 / 1,000$) and very rare ($< 1 / 10,000$). Within each frequency grouping in Table 1, adverse reactions are presented in decreasing order of seriousness.

Table 1: Travoprost 30 µg / mL eye drops, solution (IZBA eye drops)

System Organ class	Frequency	Adverse reaction
Eye disorders	Very common	ocular hyperaemia
	Common	dry eye, eye pruritus, ocular discomfort
	Uncommon	punctate keratitis, anterior chamber inflammation, blepharitis, eye pain, photophobia, visual impairment, vision blurred, conjunctivitis, eyelid oedema, eyelid margin crusting, eye discharge, dark circles under eyes, growth of eyelashes, eyelash thickening
Skin and subcutaneous tissue disorders	Uncommon	pruritus, rash

The following adverse reactions were assessed to be related with Travoprost 40 µg / ml eye drops, solution (either preserved with benzalkonium chloride or polyquaternium-1) and are classified according to the following convention : very common($\geq 1 / 10$), common ($\geq 1 / 100$ to $< 1 / 10$), uncommon ($\geq 1 / 1,000$ to $< 1 / 100$), rare ($\geq 1 / 10,000$ to $< 1 / 1,000$), very rare ($< 1 / 10,000$) and not known (cannot be estimated from the available data). Within each frequency grouping in Table 2, adverse reactions are presented in decreasing order of seriousness.

Table 2: Travoprost 40 µg / ml eye drops, solution

System Organ class	Frequency	Adverse reaction
Infections and infestations	Uncommon	herpes simplex, keratitis herpetic
Immune system disorders	Uncommon	hypersensitivity, drug hypersensitivity, seasonal allergy
Nervous system disorder	Common	headache
	Uncommon	dysgeusia, dizziness, visual field defect
Eye disorders	Very common	ocular hyperaemia, iris hyperpigmentation
	Common	punctate keratitis, anterior chamber inflammation, eye pain, photophobia, eye discharge, ocular discomfort, visual acuity reduced, vision blurred, dry eye, eye pruritus, lacrimation increased, erythema of eyelid, eyelid oedema, growth of eyelashes, eyelash discolouration
	Uncommon	corneal erosion, uveitis, keratitis, eye inflammation, photopsia, blepharitis, conjunctival oedema, halo vision, conjunctivitis, conjunctival follicles, hypoaesthesia eye, meibomianitis, ectropion, anterior chamber pigmentation, mydriasis, cataract, eyelid margin crusting, asthenopia
	Not known	macular oedema, sunken eyes
Ear and labyrinth disorders	Not known	vertigo, tinnitus
Cardiac disorders	Uncommon	heart rate irregular, palpitations, heart rate decreased
	Not known	bradycardia, tachycardia
Vascular disorders	Uncommon	blood pressure decreased, blood pressure increased, hypotension, hypertension
Respiratory, thoracic and mediastinal disorders	Uncommon	dyspnoea, asthma, respiratory disorder, oropharyngeal pain, cough, dysphonia, nasal congestion, throat irritation
	Not known	asthma aggravated
Gastrointestinal disorders	Uncommon	peptic ulcer reactivated, dry mouth gastrointestinal disorder, constipation
Skin and subcutaneous tissue disorders	Common	skin hyperpigmentation (periocular), skin discolouration
	Uncommon	dermatitis allergic, periorbital oedema, dermatitis contact, erythema, rash, hair colour changes, hair texture abnormal, hypertrichosis, madarosis
	Not known	hair growth abnormal

Musculoskeletal, connective tissue and bone disorders	Uncommon	musculoskeletal pain
General disorders and administrative site conditions	Uncommon	asthenia, malaise
Investigations	Not known	prostatic specific antigen increased

4.9 Overdose

A topical overdose of IZBA eye drops may be flushed from the eye(s) with lukewarm water.

A topical overdose is not likely to occur or to be associated with toxicity. Treatment of a suspected oral ingestion is symptomatic and supportive.

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: Antiglaucoma preparations and miotics, prostaglandin analogues. ATC code : S01EE04.

Mechanism of action

Travoprost, a prostaglandin F₂ analogue, is a highly selective full agonist which has a high affinity for the prostaglandin FP receptor, and reduces the intraocular pressure by increasing the outflow of aqueous humour via trabecular meshwork and uveoscleral pathways. Reduction of the intraocular pressure in man starts about 2 hours after administration and maximum effect is reached after 12 hours. Significant lowering of intraocular pressure can be maintained for periods exceeding 24 hours with a single dose.

Clinical efficacy and safety

In a clinical trial, patients with open-angle glaucoma or ocular hypertension treated with IZBA eye drops dosed once-daily in the evening, demonstrated intraocular pressure lowering equivalent to Travoprost 40µg / ml eye drops, solution at all on-therapy visits and time points (95 % CI within ±1.0 mmHg). The mean reduction from baseline in IOP ranged from 7.1 to 8.2 mmHg as summarised in Table 3. The mean percent reductions in IOP from baseline to each study visit and assessment time point ranged from 28.4 % to 30.7 %.

Table 3: IOP Change from Baseline (mmHg) for IZBA eye drops

Visit		8 AM	10 AM	4 PM
Week 2	Mean	-8.0	-7.3	-7.1
(N=442)	95 % CI	(-8.3, -7.7)	(-7.6, -7.0)	(-7.4, -6.8)
Week 6	Mean	-8.1	-7.4	-7.2
(N=440**)	95 % CI	(-8.4, -7.9)	(-7.6, -7.1)	(-7.5, -6.9)
Month 3	Mean	-8.2	-7.5	-7.1
(N=432**)	95 % CI	(-8.6, -7.9)	(-7.9, -7.2)	(-7.4, -6.8)

** One subject had missing data at 8 AM at Week 6 ; one had missing data at 4 PM at Month 3.

An improved safety profile has been observed for IZBA eye drops when compared to the marketed Travoprost 40 µg / ml eye drops, solution (benzalkonium chloride preserved or polyquaternium-1 preserved). The most common adverse reaction associated with both IZBA eye drops and Travoprost 40 µg / ml eye drops, solution is hyperaemia. Hyperaemia (ocular or conjunctival) was observed in 11.8 % of

patients (N = 442) exposed to IZBA eye drops compared with 14.5 % observed for patients exposed to Travoprost 40 µg / ml eye drops, solution, benzalkonium chloride preserved.

Secondary pharmacology

Travoprost significantly increased optic nerve head blood flow in rabbits following 7 days of topical ocular administration (1.4 micrograms, once-daily).

Travoprost 40 µg / ml eye drops, solution preserved with polyquaternium-1 induced minimal ocular surface toxicity, compared to eye drops preserved with benzalkonium chloride, on cultured human corneal cells and following topical ocular administration in rabbits.

5.2 Pharmacokinetic properties

Absorption

Travoprost is an ester prodrug. It is absorbed through the cornea where the isopropyl ester is hydrolysed to the active free acid. Studies in rabbits have shown peak concentrations of 20 ng / g of the free acid in aqueous humour one to two hours after topical dosing of Travoprost 40 µg / ml eye drops, solution. Aqueous humour concentrations declined with a half-life of approximately 1.5 hours.

Distribution

Following topical ocular administration of Travoprost 40 µg / ml eye drops, solution to healthy volunteers, low systemic exposure to active free acid was demonstrated. Peak active free acid plasma concentrations of 25 pg / ml or less were observed between 10 and 30 minutes post-dose. Thereafter, plasma levels declined rapidly to below the 10 pg / ml assay quantitation limit before 1 hour post-administration. Due to the low plasma concentrations and rapid elimination following topical dosing, the elimination half-life of active free acid in man could not be determined.

Biotransformation

Metabolism is the major route of elimination of both travoprost and the active free acid. The systemic metabolic pathways parallel those of endogenous prostaglandin F₂ which are characterised by reduction of the double bond in position C13-C14, oxidation of the 15-hydroxyl and -oxidative cleavages of the upper side chain.

Elimination

Travoprost free acid and its metabolites are mainly excreted by the kidneys. Travoprost 40 µg / ml eye drops, solution has been studied in patients with mild to severe hepatic impairment and in patients with mild to severe renal impairment (creatinine clearance as low as 14 ml / min). No dosage adjustment is necessary in these patients.

5.3 Preclinical safety data

In ocular toxicity studies in monkeys, administration of travoprost at a dose of 0.45 microgram, twice a day, was shown to induce increased palpebral fissure. Topical ocular administration of travoprost to monkeys at concentrations of up to 0.012 % to the right eye, twice daily for one year resulted in no systemic toxicity.

Increased palpebral fissure observed in monkeys were not seen in rabbits or in the clinical trials with travoprost products and is considered to be species specific.

Reproduction toxicity studies have been undertaken in rat, mice and rabbit by systemic route. Findings are related to FP receptor agonist activity in uterus with early embryoletality, post-implantation loss, foetotoxicity. In pregnant rat, systemic administration of travoprost at doses more than 200 times the clinical dose during the period of organogenesis resulted in an increased incidence of malformations. Low levels of radioactivity were measured in amniotic fluid and foetal tissues of pregnant rats administered ³H-travoprost. Reproduction and development studies have demonstrated a potent effect on

foetal loss with a high rate observed in rats and mice (180 pg / ml and 30 pg / ml plasma, respectively) at exposures 1.2 to 6 times the clinical exposure (up to 25 pg / ml).

Data to evaluate a potential effect on the environment are currently limited.

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Polyquaternium-1, polyoxyethylene hydrogenated castor oil 40 (HCO-40), boric acid (E284), mannitol (E421), sodium chloride, propylene glycol (E1520), sodium hydroxide and / or hydrochloric acid (to adjust pH), purified water.

6.2 Incompatibilities

In the absence of compatibility studies, this medicinal product must not be mixed with other medicinal products.

6.3 Special precautions for storage

Do not store above 30 °C.

Do not use this medicine after the expiry date which is stated on the packaging.

Discard 4 weeks after first opening.

Keep this medicine out of the sight and reach of children.

6.4 Nature and contents of container

Plastic bottle with dispensing plug and closure, containing 2.5 ml, presented in an overwrap.

6.5 Special precautions for disposal

Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

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Product Registration Holder:
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